

GUIDE 3 OF THE PARENT SERIES

Sleep & Bedtime Routines

A parent's field guide to healthy sleep habits, bedtime stalling, and night wakings in children ages 1–12 — grounded in pediatric sleep science, without harsh punishment.



BEFORE YOU START

About this guide

"One more story" turns into six. The lights go out and forty-five minutes later, someone is still calling down the hallway. If bedtime feels like the hardest hour of your day, you're not doing it wrong — and you're not alone. This guide unpacks why sleep is a genuinely hard thing to ask of a young brain, and gives you a calmer, more consistent way to build bedtime routines that actually hold.

What's inside

Why sleep is hard for kids to settle into, what's realistic to expect at each age based on current sleep-medicine guidance, a simple four-step method for calm and consistent bedtimes, ready-to-use scripts for stalling and night waking, the habits that quietly make bedtime harder, and how to build a household rhythm around sleep that doesn't rely on a fight every night.

The approach behind it

This guide is built on the American Academy of Sleep Medicine's consensus recommendations for children's sleep duration, published research on bedtime routines and sleep outcomes, and guidance from pediatric sleep specialists including Dr. Craig Canapari of Yale Medicine. Full sources are listed at the end.

Continuing the series

This is Guide 3 of the Peep & Beyond Parent Series. In Guide 1, *When Your Child Hits or Loses Their Temper*, we introduced the "upstairs/downstairs brain" and the C.A.L.M. Path. In Guide 2, *Screens & Big Feelings*, we covered the P.A.C.E. Method for ending screen time without a battle — including its screen-free wind-down advice, which pairs closely with everything in this guide. All three lean on the same idea: a child who seems to be pushing back is usually a child who needs help settling, not a child who needs a bigger consequence.

A note before you continue: This guide is educational and is not a substitute for medical advice. If you're concerned about your child's sleep, breathing at night, or daytime behavior, please speak with your pediatrician — reaching out is a sign of good parenting, not a failure of it.

CONTENTS

What you'll find inside

01	Why Sleep Is Hard for Kids	04
	The body clock, the brain, and why bedtime resistance is so common	
02	What's Realistic, By Age	05
	Sleep-medicine guidance for toddlers, preschoolers, and school-age kids	
03	The R.E.S.T. Method	07
	A simple four-step way to build calm, consistent bedtimes	
04	Scripts You Can Use	09
	Exact words by age, for stalling and for night waking	
05	Common Mistakes	10
	Habits that quietly make bedtime harder	
06	Building a Calmer Bedtime Rhythm	11
	Wind-downs, environment, and routines that do the work for you	
07	When to Reach Out	13
	Signs it's worth a conversation with a pediatrician or sleep specialist	
08	Quick Reference Sheet	14
	One page to print and keep by your child's bed	

SECTION 01

Why Sleep Is Hard for Kids

Bedtime asks a child to do something genuinely difficult: stop, alone, in the dark, while the rest of the house keeps going. A few things make that harder than it looks — and none of them mean you're doing something wrong.

A body clock still finding its rhythm

A child's circadian rhythm — the internal clock that signals when to feel sleepy and when to feel alert — is still maturing throughout childhood. The hormone that drives that sleepy feeling, melatonin, is released later and less predictably in young children than in adults, and bright light in the evening (including screens) can delay it further. That's part of why a wound-up, wide-awake child at 8pm isn't necessarily fighting sleep on purpose — their body may genuinely not feel ready yet.

Separation is the real story at bedtime

For most children, bedtime resistance isn't really about not being tired. It's about not wanting to be the one left alone. If you've read *When Your Child Hits or Loses Their Temper*, this will sound familiar: a child stalling, crying, or calling out at bedtime is usually reacting from their "downstairs brain," not calculating how to manipulate you. Treating it as a discipline problem tends to escalate it; treating it as a settling problem tends to ease it.

RESEARCH NOTE

The American Academy of Sleep Medicine's pediatric sleep consensus recommends 11–14 hours in 24 (including naps) for ages 1–2, 10–13 hours for ages 3–5, and 9–12 hours for ages 6–12 — and links falling short of these ranges with harder outcomes for attention, mood, learning, and behavior.

KEEP IN MIND

Stalling — "one more story," "I'm thirsty," "I need to tell you something" — is a normal developmental behavior, not manipulation. It gets easier with a predictable routine and a calm, consistent response, not with a bigger consequence — and an overtired child is usually harder to settle, not easier, so a consistent bedtime matters most on the hard days.

SECTION 02

What's Realistic, By Age

Sleep needs and sleep struggles both change a great deal across childhood. Here's what's realistic to expect — and what actually helps — at each stage.

TODDLERS • AGES 1–3

Toddlers need roughly 11–14 hours of sleep in 24, including naps, per sleep-medicine guidance. This is also the age of peak separation anxiety and a fast-growing sense of independence — a combination that makes bedtime a common flashpoint. Expect testing ("no bed!"), delay tactics, and the occasional full protest as your toddler pushes to see whether tonight's routine will really hold.

Naps are also shifting during this window, often dropping from two to one, which can temporarily throw bedtime off until the new rhythm settles. That's expected, not a sign anything is wrong.

What helps most: a short, very predictable routine (bath, pajamas, one or two books, lights out) in the same order every night, and a calm, boring response to stalling rather than a bigger reaction.

PRESCHOOL · AGES 3–6

Preschoolers need about 10–13 hours in 24, including naps for younger ones in this range — most children drop their daytime nap somewhere between ages 3 and 5. This is also the age of big imaginations, which means fear of the dark, monsters under the bed, and nightmares become common and very real to your child, even when nothing has actually changed at bedtime.

Stalling often becomes more verbal and more creative at this age — genuine questions, sudden worries, and elaborate reasons why "just five more minutes" is essential tonight. This is a normal stretch of testing limits, not evidence the routine has failed.

What helps most: a nightlight and a comfort item to ease dark-related fears, a short "worry" or "one question" ritual built into the routine so it doesn't spill past lights-out, and holding the same bedtime firmly even when the requests get more inventive.

SCHOOL AGE · AGES 6–12

School-age children need about 9–12 hours in 24. This is the age when homework, activities, and social life start competing for the evening — and when screens (see Guide 2) most often creep into the hour before bed. Night waking becomes less common than at younger ages, but worry-driven waking (about school, friendships, or bigger fears) can show up, especially in children prone to anxiety.

What helps most: involving your child in setting the routine so they have some ownership over it, protecting a screen-free wind-down hour, and treating a consistent bedtime as a family standard rather than something only younger siblings need.

SECTION 03

The R.E.S.T. Method

A simple four-step way to build calm, consistent bedtimes — built around what actually helps a child's body and brain wind down, not just what feels like it should.

1

Routine — the same steps, every night

The single biggest predictor of an easy bedtime isn't a strict schedule enforced in the moment — it's a short, repeatable sequence your child's body learns to recognize as the signal that sleep is coming. Bath, pajamas, teeth, one or two books, lights out, in the same order most nights, cues the brain to start easing toward sleep before you ever say goodnight. Keep it short — twenty to thirty minutes — and simple enough to hold even on a hard day.

2

Environment — a room built for sleep

A bedroom set up for sleep does a lot of the work for you: dark (a small nightlight is fine), cool, and quiet, with the same bed and comfort items night after night. Devices are best left outside the room during sleep — the light and stimulation both work against the body's natural release of melatonin, which is part of why so many children seem to "wake back up" right after screen time ends.

3

Soothe — meet resistance with calm, not conflict

When stalling or big feelings show up anyway — the tenth glass of water, the sudden urgent story, the tears the moment you stand to leave — meet it the way you would any other hard moment: get low, soften your voice, and name what's happening. "It's hard to be the one who has to fall asleep first." This isn't about adding more time; it's about helping your child's downstairs brain settle enough to actually let go of the day, then returning calmly to the routine.

4

Time — hold the line, gently and consistently

Keep the same bedtime and wake time on most days, including weekends — irregular sleep timing has been linked to worse attention and behavior even when total hours look fine on paper. If your child asks for "one more thing," a single warm "I love you, I'll see you in the morning," said once and followed through, works better than an ever-lengthening negotiation.

RESEARCH NOTE

A study by pediatric sleep researcher Dr. Jodi Mindell found that instituting a consistent nightly bedtime routine improved sleep onset, reduced night wakings, and lifted overall sleep quality within just days — with benefits growing the more consistently the routine was used, a "dose-dependent" effect that held true across very different routines and cultures.

SECTION 04

Scripts You Can Use

A few ready phrases, by age, for bedtime stalling and for the middle-of-the-night wake-up.

AGE	IF BEDTIME STALLS	IF THEY WAKE AT NIGHT
Toddler 1–3	"All done with books. Lights are sleepy now." <i>(Said the same simple way every night.)</i>	"I'm here. It's still sleep time." Keep the room dark, your voice quiet, the visit brief.
Preschool 3–6	"Two books, one song, then lights out — that's our plan tonight."	"You're safe. It's still nighttime. Let's lie back down together."
School age 6–12	"Lights out at the usual time — reading light's okay until then, same as always."	"I hear you. Take three slow breaths — I'll check back in a few minutes."

TIP

Say it once, calmly, and follow through — keeping night-waking visits short, quiet, and low-light helps your child fall back asleep faster and avoids accidentally turning 2am into playtime.

SECTION 05

Common Mistakes

Every one of these is an understandable response to an exhausting hour — most parents have reached for all of them. Here's what tends to help less than it seems like it should, and what to try instead.

Tends to backfire

- A different bedtime and routine every night
- Screens or high-energy play right up until lights out
- Negotiating "just one more" again and again
- Threatening to cancel tomorrow's fun as punishment
- Rushing in at the very first sound overnight
- A long lecture in response to stalling
- Letting weekend bedtime slide by hours

Works better

- The same bedtime and wake time most days, weekends included
- A calm, screen-free wind-down the last 30–60 minutes
- One warm goodnight, said once, then holding the limit
- Brief, boring, low-light check-ins if they wake at night
- Naming the feeling, then returning to the routine
- The same quiet response, night after night
- A short buffer on weekends, not a total reset

RESEARCH NOTE

Research on school-age children's sleep patterns has linked night-to-night bedtime variability with poorer behavioral and attention outcomes — independent of total sleep duration. Consistency in timing appears to matter almost as much as how many hours a child sleeps.

SECTION 06

Building a Calmer Bedtime Rhythm

The goal isn't a perfect night with zero resistance — it's a rhythm predictable enough that most nights don't need a battle at all.

Start the wind-down before the fight would start

Begin easing the pace thirty to forty-five minutes before your target bedtime — dimmer lights, quieter voices, calmer activities — rather than asking your child to go from full energy to lights-out in five minutes. A gradual slide down is far easier for a young nervous system than an abrupt stop.

Let your child help write the routine

Especially for school-age kids, a small amount of real choice — which book, which pajamas, what order the last two steps happen in — builds genuine buy-in. A routine your child helped shape is one they're more likely to cooperate with, even on nights they'd rather stay up.

Watch what's feeding the resistance

Hunger, a skipped or too-late nap, and overstimulation late in the day all make bedtime harder, sleep struggles included. Before assuming a hard night is "just bedtime being bedtime," it's worth checking the same basics from Guide 1: has your child eaten enough, had some downtime, and avoided getting overtired today?

Keep the hour before bed screen-free

FROM GUIDE 2 — THIS STILL APPLIES

No screens in the hour before bed is one of the most consistently recommended habits in pediatric sleep guidance — both for the light delaying melatonin, and because ending screen time itself can spark exactly the kind of meltdown that makes settling down even harder. See *Guide 2: Screens & Big Feelings* for the full P.A.C.E. Method on ending screen time without a fight.

Build in a worry check for older kids

For school-age children whose stalling is really anxiety in disguise, a short, structured "worry time" earlier in the evening — naming one thing on their mind and writing it down or telling you about it — can prevent it from resurfacing the moment the lights go out.

Model calm around sleep yourself

Children notice how the adults around them treat sleep, too. Keeping your own tone matter-of-fact about bedtime, rather than dreading or announcing it as a battle in advance, quietly shapes how your child expects the whole hour to go.

SECTION 07

When to Reach Out

Some bedtime resistance and occasional night waking are a normal, expected part of childhood. A few patterns are worth a conversation with your pediatrician or a pediatric sleep specialist:

- Loud snoring, gasping, or pauses in breathing during sleep — possible signs of sleep apnea
- Persistent, severe insomnia that isn't improving after several weeks of a genuinely consistent routine
- Extreme daytime sleepiness or trouble staying awake, despite what looks like enough time in bed
- Frequent or intense night terrors, or sleepwalking that puts your child at risk
- Sleep trouble tied to significant anxiety, or a major change (a new sibling, a move, a loss) that isn't easing over time
- You feel unable to get your child to sleep or stay asleep at all, despite consistent effort

PLEASE REMEMBER

Reaching out for support is not a sign that you've failed as a parent — it's one of the most effective things you can do for your child. A pediatrician or pediatric sleep specialist can rule out underlying issues and help build a plan suited to your specific child and family.

SECTION 08 • PRINT & KEEP

Quick Reference

Keep this page by your child's bed for the next hard bedtime.

The R.E.S.T. Method

1. **Routine** — same steps every night
2. **Environment** — dark, cool, screen-free
3. **Soothe** — name the feeling, then return
4. **Time** — same bedtime, held consistently

Say this

"Lights are sleepy now."
"It's hard to be the one who falls asleep first."
"I love you, I'll see you in the morning."

Skip this

Endless "one more" negotiating · long lectures · screens near bedtime · a different routine every night

Remember

Overtired makes bedtime harder, not easier.
Consistency beats a perfect routine. A slow wind-down beats a sudden lights-out.

Reach out if...

Snoring or pauses in breathing during sleep · severe insomnia lasting weeks despite a consistent routine · extreme daytime sleepiness · frequent, risky night terrors or sleepwalking → talk to your pediatrician or a pediatric sleep specialist.

SOURCES & FURTHER READING

Where this guide comes from

American Academy of Sleep Medicine. Pediatric sleep duration consensus recommendations by age, and the Child Sleep Duration Health Advisory.

Mindell, J.A., et al. Research on nightly bedtime routines and their dose-dependent association with sleep outcomes in infants and young children.

Sleep Foundation. Guidance on children's bedtime routines and sleep environment.

Canapari, C., MD, Yale Medicine. Pediatric sleep-medicine guidance on bedtime resistance and night wakings.

Peer-reviewed research on sleep timing variability. Studies linking irregular bedtimes with behavioral and attention outcomes in school-age children.

American Academy of Pediatrics. Guidance on screen use and children's sleep — see also Guide 2: Screens & Big Feelings.

This guide is part of the Peep & Beyond Parent Series, created to give parents clear, research-informed support for the everyday moments of raising kids — including the ones that happen after the lights go out. It is educational in nature and does not replace individualized medical or mental-health care.

With you, one calm bedtime at a time.

— Peep & Beyond